1. Introduction

Rehabilitation services at Hope Hospital aim to enhance and restore functional ability and quality of life to those with physical impairments or disabilities. The services are aligned with NABH COP.18 standards and support the patient's recovery through coordinated, multi-disciplinary care.

2. Objectives

- To provide safe, consistent, and collaborative rehabilitative care.
- To support patients in regaining mobility, independence, and improved quality of life.
- To reduce post-operative or post-trauma complications through early interventions.
- To offer prehabilitation for surgical patients, where applicable.

3. Scope of Services

The rehabilitation services include but are not limited to the following areas:

Service	Details
Physiotherapy	Post-operative care, musculoskeletal rehab, stroke rehab, pain management
Occupational Therapy	Training in daily living activities, functional independence (Outsourced or referred)
Speech Therapy	Rehabilitation post-stroke or trauma (On-call via MoU)
Neuro-rehabilitation	For patients with stroke, TBI, spinal cord injuries
Cardiac Rehabilitation	Post-angioplasty or post-CABG rehab plans (On referral)
Pulmonary Rehabilitation	Breathing exercises for COPD, asthma patients
Geriatric Rehabilitation	Mobility and strength-building for elderly
Paediatric Rehabilitation	Support for children with developmental delays (Outsourced)

4. Staffing

The rehab team consists of:
- Qualified Physiotherapists (Full-time)
- Visiting Occupational and Speech Therapists
- Nursing staff trained in mobility support
- Consultants from Orthopaedics, Neurology, Cardiology

5. Process Flow

1. Referral by treating consultant
2. Initial assessment and goal setting by rehab team
3. Consent and education to patient/family
4. Daily or alternate day therapy as per need
5. Re-assessment every 7 days or before discharge

6. Handover to home-care or external rehab centre, if required

6. Facilities and Equipment

- Parallel bars, exercise bikes, weights, TENS, IFT machines
- Adjustable therapy couches, walker, crutches
- Wheelchairs, resistance bands, tilt table

7. Quality Assurance

- Daily progress documentation
- Monthly rehab audit (sample 5-10 cases)
- Patient feedback system
- Incident reporting mechanism for falls/injuries
- CME sessions for rehabilitation staff quarterly

8. Exclusions

- Invasive rehabilitation procedures (e.g., botox for spasticity)
- In-patient psychiatric rehabilitation
- Long-term institutional rehab services

9. Review and Updates

This scope document shall be reviewed annually or earlier based on changes in service capability, staff, or compliance requirements.